

METROPOLITAN GENERAL HOSPITAL

2100 Healing Way, Boston, MA 02115 | (617) 555-0800 | www.metrogen.org

DISCHARGE SUMMARY

Patient Name: Hannah Kim	DOB: December 18, 1974 (Age 71)
MRN: MRN-5232413	Gender: Male
Admission Date: May 31, 2025	Discharge Date: April 06, 2025 (8 days)
Attending Physician: Dr. Rajesh Patel, MD — Endocrinology	Insurance: BlueCross BlueShield #BCB-779-00234

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Acute ST-Elevation Myocardial Infarction (STEMI) — Anterior Wall	I21.09	Resolved
Osteoarthritis of Knee	M17.9	Chronic / Pre-existing
Acute Kidney Injury, Stage 1	N17.9	Resolved
Hypothyroidism	E03.9	Chronic / Pre-existing
Gastroesophageal Reflux Disease (GERD)	K21.9	Chronic / Pre-existing

HOSPITAL COURSE

Hannah, a 71-year-old male with a known history of Type 2 Diabetes and Hypertension, presented to the Emergency Department on May 31, 2025 via ambulance with a 2-hour history of severe substernal chest pain radiating to the left arm, diaphoresis, and nausea. Initial ECG demonstrated ST-segment elevation in leads V1–V4 consistent with anterior STEMI. Troponin-I on arrival was critically elevated at 20.1 ng/mL. The patient was taken emergently to the cardiac catheterization laboratory where coronary angiography revealed 87% occlusion of the left anterior descending (LAD) artery. Successful percutaneous coronary intervention (PCI) was performed with deployment of a drug-eluting stent (Xience Skypoint 3.0 x 28mm). Post-procedural TIMI flow was grade 3. The patient was transferred to the Cardiac Care Unit (CCU) for monitoring. Serial echocardiography on day 2 showed an ejection fraction of 39%, mild anterior wall hypokinesis. Mild acute kidney injury (Creatinine 1.7 mg/dL) was noted, attributed to contrast nephropathy; this resolved with IV hydration by day 4. Blood glucose remained elevated throughout admission and endocrinology was consulted to optimize insulin regimen. The patient remained hemodynamically stable throughout and was discharged on day 8 in stable condition with close outpatient follow-up arranged.

VITALS AT DISCHARGE

BP	HR	SpO2	Temp	RR	Weight
117/83 mmHg	73 bpm	98% (RA)	98.8°F	20/min	86 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Aspirin	81 mg	Once daily	Indefinite
Clopidogrel (Plavix)	75 mg	Once daily	12 months
Atorvastatin	80 mg	Once at bedtime	Indefinite
Metoprolol Succinate	50 mg	Once daily	Indefinite
Lisinopril	10 mg	Once daily	Indefinite
Insulin Glargine (Lantus)	20 units	Once daily at bedtime	Ongoing — follow with Endo
Metformin	500 mg	Twice daily with meals	Ongoing

FOLLOW-UP PLAN

Cardiology: Dr. Forsythe — 1 week post-discharge. Repeat Echo at 6 weeks.
Endocrinology: Dr. Patel — 2 weeks post-discharge for diabetes management.
Primary Care: Dr. Kim — 4 weeks. Renal function panel to confirm AKI resolution.
Cardiac Rehab: Enrollment arranged — starts July 01, 2025 at MetroGen Rehab Center.
Labs: BMP, HbA1c, Lipid Panel — 2 weeks post-discharge.

Attending Physician Signature: _____
Dr. Rajesh Patel, MD — Endocrinology | License #: MA-MD-49949 | Date: April 06, 2025
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